

Critical Summary

Patient Name: [Patient Name]

MRN: [Patient MRN]

DOB: (Date of Birth)

Blood Type: Not specified

Allergies: Benicillin (rash), Fish (shortness of breath)

Chronic Conditions: Type 2 Diabetes, Hypertension

Current Medications: Metformin, Lisinopril

Admitting Problem: Chest pain for 24 hours, starting around 9:00 PM last night. Pain described as heavy, squeezing pressure in the center of the chest, radiating to the left arm. Shortness of breath and diaphoresis associated with the chest pain. Denies prior similar episodes, but reports a similar episode approximately 6 months ago which resolved spontaneously. Denies other significant symptoms.

Assessment: Possible cardiac etiology for chest pain, given patient's description of the pain and associated symptoms. Consider cardiac ischemia or other cardiac event.

Plan: STAT ECG ordered. STAT cardiac enzymes ordered. ABG to assess for respiratory compromise. Oxygen saturation to be monitored. Patient to remain on continuous cardiac monitoring. Patient to be placed on a cardiac monitor.